

• A SAVE A LEG, SAVE A LIFE FOUNDATION INITIATIVE

Time Out Before Amputation



No amputation without a vascular test first. TOBA is a simple safety pause that makes sure every patient's blood flow is checked before a limb is removed.

UP TO

1 in 2

non-traumatic lower-limb amputations in the U.S. happen **without any prior vascular testing**. That is an estimated **48,000 to 74,000 people every year** who lose a toe, foot, or leg before anyone checks whether restoring blood flow could have saved it.

“How is this acceptable? What other body parts can be removed surgically without prior diagnostic testing? The final decision regarding the need for amputation should be based on objective findings, not subjective opinion.”

THE SAVE A LEG, SAVE A LIFE FOUNDATION

What TOBA asks for

Before any non-traumatic lower-extremity amputation, the care team calls a time out and confirms that **vascular studies have been done, the results are in front of the surgeon, and every option to restore blood flow has been considered.**

IN PLAIN WORDS

Most amputations happen because a leg or foot is not getting enough blood. A vascular test measures that blood flow. If the flow can be restored, the limb may be saved, or a smaller surgery may be possible.

THINK OF IT THIS WAY

You can't have a tooth pulled without an X-ray. You shouldn't lose a leg without a vascular study.



thesalsal.org/toba

Scan to learn more, download the patient flyer and clinician checklist, and watch the on-demand TOBA session.

A gap in care we can close

150,000 non-traumatic lower-extremity amputations in the U.S. each year	85% of those amputations are considered avoidable	~100% receive vascular testing first when a vascular surgeon is involved	42–50% fewer major amputations with multidisciplinary limb-salvage teams
---	---	--	--

Source: Swoboda L, Bell D, Bauer K, Clayton S. Time Out Before Amputation. *Advances in Skin & Wound Care*. 2026;39(5):234–235, and the studies it cites.

How the time out works

- 01 Pause before the procedure**
A brief, structured stop for any patient being considered for amputation, modeled on the surgical safety time out hospitals already use.
- 02 Confirm the vascular studies**
Ankle-brachial index, toe pressures, arterial ultrasound, CT angiography, or other vascular imaging: done, displayed, and reviewed.
- 03 Bring in vascular expertise**
A vascular surgeon or interventionalist evaluates whether blood flow can be restored.
- 04 Decide on objective findings**
If testing is missing, the team stops and gets it before any irreversible step.

What hospitals can do now

- Adopt TOBA as part of surgical safety and limb preservation protocols.
- Make vascular test results visible to the surgeon before the procedure.
- Track vascular testing rates before every amputation.
- Build a multidisciplinary limb preservation team: vascular, wound care, podiatry, and primary care.
- Partner with SALSAL to make TOBA the standard of care.

PUBLISHED AND PRESENTED

TOBA is described in **Advances in Skin & Wound Care** (June 2026) and was presented as a research poster, *A Novel Strategy for Limb Salvage*. Early results from a pilot hospital in North Carolina showed the protocol could be put into practice, along with a reduction in amputation severity. Further research is needed.

DOI: [10.1097/ASW.0000000000000450](https://doi.org/10.1097/ASW.0000000000000450)



Join the movement: thesalsal.org/toba

The Save a Leg, Save a Life Foundation is a 501(c)(3) nonprofit working to reduce preventable amputations through education, advocacy, and policy change. This information is for educational purposes only and is not intended to replace professional medical advice. Always consult your healthcare provider.

